INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Atezolizumab + nab-paclitaxel

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00688
Version 2a

Atezolizumab 840 mg is given on Days 1 and 15, before nab-paclitaxel 100 mg/m² on Days 1, 8 and 15 of each 28-day cycle. Nab-paclitaxel is planned for up to six cycles; extension needs consultant review. Atezolizumab may continue after chemotherapy stops.

Treatment stages

01 **Combination treatment**
28-day reference

02 **Atezolizumab continuation if selected**
28-day reference

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Atezolizumab + nab-paclitaxel
National Cancer Control Programme (NCCP) 00688

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Fever, new cough or breathlessness, diarrhoea, severe rash, jaundice, unusual weakness, marked thirst, severe headache or confusion needs prompt advice, including after treatment ends.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

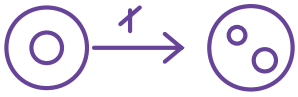
Your medicines and what they do

Control advanced cancer and symptoms while balancing side effects and treatment burden.

PD-L1 CHECKPOINT IMMUNOTHERAPY

Atezolizumab

Atezolizumab



Removes an immune checkpoint signal

Atezolizumab blocks PD-L1 signalling that can restrain immune cells. This may help the immune system attack cancer, but it can also inflame healthy organs. Immune effects can begin during treatment or after it has stopped.

PD-L1 blocked → Immune inhibition reduced → Immune cells can attack cancer

What to watch for

Report new cough, diarrhoea, jaundice, severe rash, unusual weakness, headache, thirst or changes in urination promptly.

CHEMOTHERAPY

Nab-paclitaxel

Albumin-bound paclitaxel · Abraxane



Disrupts cell division by stabilising microtubules

This formulation carries paclitaxel with albumin, a protein. It interferes with the structures cells use to divide. Its dosing and allergy precautions differ from ordinary paclitaxel.

Albumin-bound medicine → Microtubules held stable → Cell division disrupted

What to watch for

Report numbness, tingling, weakness or problems with daily tasks before the next dose. Tell staff about an infusion reaction; new cough or breathlessness needs prompt assessment.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment?

This combination brings checkpoint immunotherapy together with albumin-bound paclitaxel for a defined first-line triple-negative breast population. The validated PD-L1 test, previous treatment, autoimmune history, organ health and patient priorities affect suitability.

Your treatment setting

Previously untreated unresectable locally advanced or metastatic triple-negative breast cancer with eligible PD-L1 expression

Your exact schedule

Atezolizumab 840 mg is given on Days 1 and 15, before nab-paclitaxel 100 mg/m² on Days 1, 8 and 15 of each 28-day cycle. Nab-paclitaxel is planned for up to six cycles; extension needs consultant review. Atezolizumab may continue after chemotherapy stops.

Alternatives

Discuss biomarker-directed, endocrine, chemotherapy and supportive-care options appropriate to this setting, including the option not to proceed.

Why this treatment may be chosen

Discuss how this evidence and the available alternatives apply to your own situation.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Treatment and review

Treatment stages

01 **Combination treatment**
28-day reference

02 **Atezolizumab continuation if selected**
28-day reference

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

28-DAY REFERENCE

Combination treatment

28-day interval

Atezolizumab Days 1/15 and nab-paclitaxel Days 1/8/15. Review the chemotherapy component after up to six cycles.

28-DAY REFERENCE

Atezolizumab continuation if selected

28-day interval

Immunotherapy may continue after nab-paclitaxel stops; immune checks and symptom reporting continue.

Your team confirms the active medicines, treatment days, dose changes and next review.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm completion.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00688 v2a · official regimen · NICE NG101 · bone health and follow-up · ASCO · biomarkers in metastatic breast cancer · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

Atezolizumab + nab-paclitaxel: treatment and checks

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	During treatment	Change / completion	Follow-up
Treatment plan NCCP 00688	SP142 IC≥1%; no prior advanced chemotherapy	D1 / D15 both medicines	D8 nab-paclitaxel only	Nab up to 6 cycles; atezolizumab may continue	Immune / disease review
Blood / organ function NCCP 00688	full blood count (FBC), renal, liver, glucose	Check before cycle	FBC / renal / liver / glucose each cycle	Confirm Day 8/15 count decisions	Review continuing abnormalities
Immune / thyroid NCCP 00688	thyroid function tests (TFT), cortisol, hepatitis B virus (HBV) / hepatitis C virus (HCV)	TFT Cycle 1 Day 1	TFT every second cycle	Continue when chemotherapy stops	Immune symptoms may occur later
Cancer review METASTATIC · MARKERS	Confirm setting	Agree review plan	Response / symptom review	Next treatment / completion	Agreed surveillance

What each check is for

1 Before treatment

Before the first dose

Previously untreated unresectable locally advanced or metastatic triple-negative breast cancer with eligible PD-L1 expression. Baseline FBC, kidney/liver function, glucose, thyroid function, hepatitis B/C testing and morning cortisol. FBC, kidney/liver tests and glucose before each cycle; check the count results needed for Day 8 and Day 15 decisions.

Ask / act: Confirm eligibility, test results and medicine list.

2 Other checks

At the specified review points

Thyroid function at Cycle 1 Day 1 and every second cycle. Review immune symptoms throughout and after treatment. Cardiac imaging is based on history/prior anthracyclines or clinical concern; there is no routine HER2-style echo schedule.

Ask / act: Confirm who arranges and reviews each check.

3 Support and practical plan

At each treatment or phase change

Separate combination, Day 8 chemotherapy-only and immunotherapy-only support; maintain immune and blood-count monitoring.

Ask / act: Choose the matching support plan; confirm active medicines.

4

Response and follow-up

Agreed review points

Agree the scan type and review interval for the known disease sites. Review response, symptoms and toxicity together; new or worsening symptoms may need an earlier assessment. Keep the agreed follow-up and report persistent or new symptoms between visits.

Ask / act: Review symptoms, results and patient priorities.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your treatment and access plan

STEP 1

Before treatment

Baseline FBC, kidney/liver function, glucose, thyroid function, hepatitis B/C testing and morning cortisol. FBC, kidney/liver tests and glucose before each cycle; check the count results needed for Day 8 and Day 15 decisions.

STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. A port is not automatically required; insertion and removal are separate procedures with individual instructions.

STEP 3

On the treatment day

Atezolizumab is given first, then nab-paclitaxel; Day 8 has nab-paclitaxel only.

STEP 4

After the visit

No routine take-home infusion pump is required for this regimen. Keep line-care, urgent-contact and next-review instructions.

Visit length

Atezolizumab: 60 minutes initially, then 30 minutes if tolerated; nab-paclitaxel: 30 minutes. Full visits also include preparation, review and observation.

Line care

A PICC or port has its own care plan. Report fever, redness, swelling, pain, leakage or line damage; do not remove or adjust it yourself.

Treatment changes

A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

Support for this visit

Separate combination, Day 8 chemotherapy-only and immunotherapy-only support; maintain immune and blood-count monitoring.

Checks across treatment

Your team arranges tests and reviews the results before treatment decisions.



Blood and organ checks

Other treatment-specific checks

Thyroid function at Cycle 1 Day 1 and every second cycle. Review immune symptoms throughout and after treatment. Cardiac imaging is based on history/prior anthracyclines or clinical concern; there is no routine HER2-style echo schedule.



Cancer review and follow-up

Agree the scan type and review interval for the known disease sites. Review response, symptoms and toxicity together; new or worsening symptoms may need an earlier assessment.

Bone health

Bone-health assessment follows endocrine therapy, ovarian suppression, menopause and other fracture risks. It has its own monitoring plan.

The wider treatment plan

Review response, toxicity and priorities to decide whether to continue, change or stop. Supportive and palliative care can accompany anticancer treatment.



Medicines review

Check new prescriptions, over-the-counter medicines and supplements with the oncology pharmacist before taking them.

After treatment

Keep the agreed follow-up and report persistent or new symptoms between visits.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Atezolizumab + nab-paclitaxel · 28-day reference

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Treatment visit · Day 1	 **Day 1** Atezolizumab then nab-paclitaxel
Treatment visit · Day 8	 **Day 8** Nab-paclitaxel only
Treatment visit · Day 15	 **Day 15** Atezolizumab then nab-paclitaxel
Blood / infection	 **Days 1–28** Fever or sudden illness: seek advice
Numbness / tingling	 **Days 1–28** Report changes before the next dose
Fatigue	 **Days 1–28** May build across treatment
Immune symptoms	 **Days 1–28** Can arise at any time, including after treatment

Timing varies: Symptoms can occur on any day, including days without treatment. The bands are orientation, not predicted symptom-free periods.

Treatment-day changes: A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

Why these bands stay broad: These broad symptom bands are educational possibilities, not measured risk curves. Infusion days are the planned schedule; delays require a new clinical plan. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00688 v2a · official regimen ↗

There are no guaranteed "safe days". Fever, shivers, sudden illness, severe breathing changes or chest pain need prompt assessment on any day.

Between-treatment notes

Day 1–28: Reference cycle

Atezolizumab 840 mg is given on Days 1 and 15, before nab-paclitaxel 100 mg/m² on Days 1, 8 and 15 of each 28-day cycle. Nab-paclitaxel is planned for up to six cycles; extension needs consultant review. Atezolizumab may continue after chemotherapy stops. Follow your written appointment and medicine plan. A calendar does not authorise a dose.

Support around the medicines

1 **Know the treatment day**

Day 8 is nab-paclitaxel only. Confirm which medicines are due at every visit.

2 **Immune symptoms**

Report new symptoms promptly, even after treatment ends. Do not assume diarrhoea or breathlessness is an ordinary chemotherapy effect.

3 **Nerves and blood counts**

Report numbness before the next dose; fever or unusual bleeding needs urgent advice.

4 **Medicines and vaccines**

Discuss steroids, new medicines and vaccinations with the team.

Questions worth asking

Who arranges the checks and explains the results?

Agree the due window, booking team, result reviewer and contact route if a test or appointment is missing. This page does not record completed checks.

Try explaining the plan back: Which medicines and doses are prescribed, when are they due, which checks are needed, and what would make you call the team today?

Atezolizumab

- Immune inflammation of lungs, bowel, liver or kidneys
- Thyroid, adrenal or pituitary problems and diabetes
- Rare serious heart, nerve or muscle inflammation
- Infusion reactions and severe skin reactions

Nab-paclitaxel

- Numbness / tingling or weakness, sometimes lasting
- Low blood counts, infection and sepsis
- Hair loss, fatigue and muscle / joint aches
- Infusion reactions; uncommon serious lung inflammation

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Days 1 and 15 · atezolizumab + nab-paclitaxel

Active medicines: Atezolizumab + Nab-paclitaxel

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Before each administered chemotherapy dose Review QT risk, constipation, liver function and any steroid already prescribed.
Metoclopramide	10 mg orally up to three times daily as needed	Rescue duration to be confirmed Review contraindications, renal function and interactions.

Additional phase-specific support

- No routine corticosteroid or antihistamine premedication is required by the source for these medicines. Prior infusion reaction needs an individual plan.
- Give atezolizumab first, then nab-paclitaxel. Do not substitute solvent-based paclitaxel premedication.
- Review immune symptoms, count results and neuropathy before administration.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Day 8 · nab-paclitaxel only

Active medicines: Nab-paclitaxel

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Before each administered chemotherapy dose Review QT risk, constipation, liver function and any steroid already prescribed.
Metoclopramide	10 mg orally up to three times daily as needed	Rescue duration to be confirmed Review contraindications, renal function and interactions.

Additional phase-specific support

- Atezolizumab is not scheduled on Day 8.
- No routine solvent-taxane steroid premedication. Check the current count and neuropathy decision.
- Continue to assess immune symptoms from previous atezolizumab even on chemotherapy-only days.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Atezolizumab alone · after chemotherapy stops

Active medicines: Atezolizumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine chemotherapy antiemetic or nab-paclitaxel premedication.
- Keep the source thyroid/blood monitoring and immune-toxicity review; stopping chemotherapy does not end immune risk.
- Confirm continuation has been authorised and use the atezolizumab schedule in this source.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

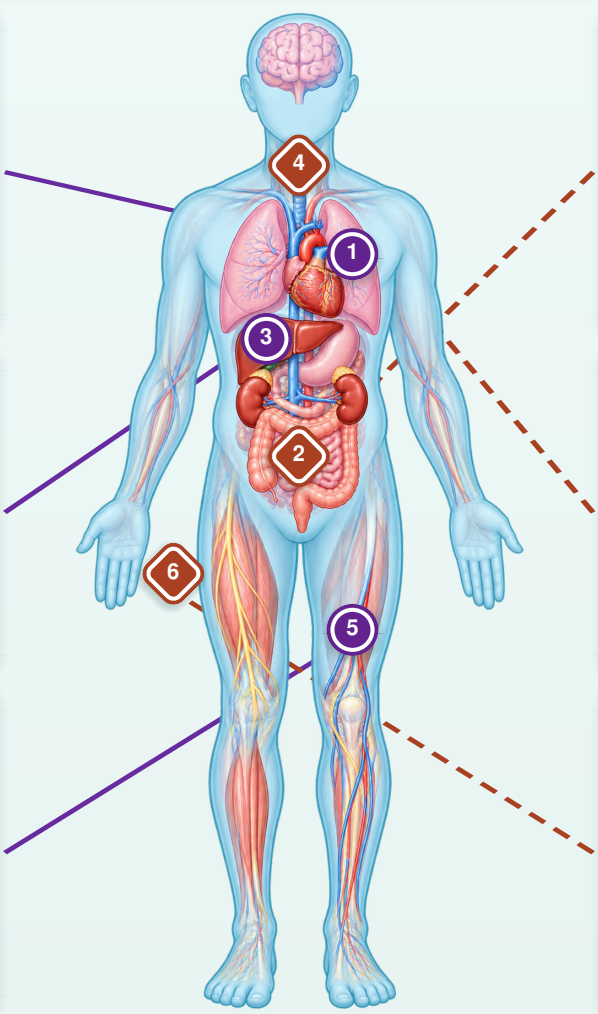
Atezolizumab + nab-paclitaxel

Atezolizumab 840 mg is given on Days 1 and 15, before nab-paclitaxel 100 mg/m² on Days 1, 8 and 15 of each 28-day cycle. Nab-paclitaxel is planned for up to six cycles; extension needs consultant review. Atezolizumab may continue after chemotherapy stops.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Breathing

Atezolizumab

New cough or breathlessness may reflect immune inflammation.

2 Bowel

Atezolizumab / nab-paclitaxel

Report diarrhoea early, especially pain or blood in stool.

3 Liver

Atezolizumab

Jaundice or dark urine needs prompt review.

4 Hormones / energy

Atezolizumab

Marked weakness, thirst, headache or dizziness needs assessment.

5 Blood / infection

Nab-paclitaxel

Fever, bruising or sudden illness: contact oncology.

6 Hands / feet

Nab-paclitaxel

Report numbness or tingling before another dose.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Selected medicine-specific effects; this is not a complete list. Report new or persistent symptoms even after treatment stops.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Treatment review · confirm active medicines · copy this sheet for another cycle of the same phase.

Atezolizumab · PD-L1 checkpoint immunotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Skin / nail changes	□ None	□ Mild change: report	□ Pain or daily tasks affected: call	□ Spreading infection or blistering: urgent review	□ Severe illness / collapse: emergency help
Headache	□ None	□ Mild and short-lived: record	□ New / persistent headache: call	□ Severe headache / visual change: urgent help	□ Sudden worst headache / weakness: 112/999

Nab-paclitaxel · Chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Numbness / tingling	□ None	□ Mild change; usual tasks unaffected	□ Limits buttons, writing or walking: call	□ Washing, dressing or walking difficult: urgent review	□ Sudden major weakness / paralysis: emergency help
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Muscle / joint pain	□ None	□ Mild aches; normal activity	□ Pain limits tasks / sleep: call	□ Pain limits self-care: urgent review	□ Sudden severe deterioration: emergency help
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Carry your treatment details

Print, cut out and fold. Follow the contact plan from your own hospital.

SACTCHECK · SACT TREATMENT

Atezolizumab + nab-paclitaxel

NCCP 00688 v2a · 28-day cycle

Atezolizumab 840 mg is given on Days 1 and 15, before nab-paclitaxel 100 mg/m² on Days 1, 8 and 15 of each 28-day cycle. Nab-paclitaxel is planned for up to six cycles; extension needs consultant review. Atezolizumab may continue after chemotherapy stops.

1	2	3	4	5	6

Your contact plan

Oncology team: Insert numbers here
Out of hours: Insert numbers here.

Evenings / overnight: instructed out-of-hours pathway or nearest ED.
Emergency: 112 / 999.

Other hospital / own contact: _____

QR: https://sactcheck.com/patient/00688/ · no patient information.

Draft for review · SACTCheck v0.84.19

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00688 v2a · official regimen**
   https://healthservice.hse.ie/documents/6530/688_v2a_Atezolizumab_nab-PACLitaxel.pdf
2. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
3. **ASCO · biomarkers in metastatic breast cancer**
   https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
4. **ESMO metastatic breast cancer guideline · 2026**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
5. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
6. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
7. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
8. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
9. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
10. **NOGG · section 7, aromatase inhibitors and fracture risk**
    https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
11. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf